

NEWBORN (BIRTH, PRE-DISCHARGE)

Well-child / well-adolescent preventive visit • Infancy • Visit 1 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Birth wt
- ☐ length
- ☐ head circumference
- ☐ gestational age

PHYSICAL EXAMINATION

- ☐ Full exam
- ☐ hips (Ortolani/Barlow)
- ☐ red reflex
- ☐ cardiac exam
- ☐ femoral pulses
- ☐ SpO2 (CCHD)

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ BCG
- ☐ OPV-0
- ☐ Hep B-1 (within 24 h)

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Newborn screening: TSH (minimum)
- ☐ expanded NBS (CAH/17-OHP, G6PD, GALT, PKU, biotinidase) where available
- ☐ CCHD pulse oximetry

VISION / HEARING / DENTAL

- ☐ Newborn hearing (OAE/BERA) done
- ☐ red reflex checked
- ☐ Palate/gums inspected
- ☐ check for oral thrush

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Newborn behaviour, tone & feeding cues observed
- ☐ Maternal wellbeing & bonding noted
- ☐ postpartum depression risk asked

ANTICIPATORY GUIDANCE GIVEN

- ☐ Exclusive breastfeeding counselled
- ☐ warmth/thermal care
- ☐ danger signs explained
- ☐ safe sleep (back, firm surface, no soft bedding)
- ☐ birth registration

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

3-5 DAYS

Well-child / well-adolescent preventive visit • Infancy • Visit 2 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight (assess loss <10% of birth wt)
- ☐ jaundice level

PHYSICAL EXAMINATION

- ☐ Weight trend
- ☐ hydration
- ☐ cord
- ☐ jaundice (Kramer/TcB)
- ☐ tone

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Complete any missed birth doses

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Bilirubin ONLY if clinically jaundiced (risk-based)
- ☐ repeat NBS if first sample missed

VISION / HEARING / DENTAL

- ☐ Confirm newborn hearing screen completed

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Feeding adequacy & alertness assessed
- ☐ Maternal EPDS if any concern

ANTICIPATORY GUIDANCE GIVEN

- ☐ Latch & feeding support given
- ☐ jaundice warning signs
- ☐ when to return urgently explained

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

1 MONTH

Well-child / well-adolescent preventive visit • Infancy • Visit 3 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ length
- ☐ head circumference
- ☐ plotted on WHO chart

PHYSICAL EXAMINATION

- ☐ Fontanelle
- ☐ eyes/red reflex
- ☐ heart
- ☐ hips
- ☐ genitalia
- ☐ social smile emerging

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Confirm Hep B-2 given with 6-wk combo (if applicable)

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ Eyes tracked
- ☐ startle to sound checked

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Surveillance: fixes & follows
- ☐ responds to sound
- ☐ Maternal postpartum depression screen (EPDS)

ANTICIPATORY GUIDANCE GIVEN

- ☐ Feeding on demand
- ☐ tummy time
- ☐ safe sleep
- ☐ no honey
- ☐ counsel for 6-week immunisation

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

6 WEEKS

Well-child / well-adolescent preventive visit • Infancy • Visit 4 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ length
- ☐ head circumference
- ☐ plotted

PHYSICAL EXAMINATION

- ☐ General exam
- ☐ hips
- ☐ eyes
- ☐ heart
- ☐ developmental surveillance

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ DTaP-1 (or DTaP)
- ☐ IPV-1
- ☐ Hib-1
- ☐ Hep B-2
- ☐ PCV-1
- ☐ Rotavirus-1

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ Visual tracking & sound response checked

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Fix & follow
- ☐ cooing noted
- ☐ Maternal EPDS

ANTICIPATORY GUIDANCE GIVEN

- ☐ Post-vaccination fever advice
- ☐ feeding
- ☐ developmental play
- ☐ fall prevention

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

10 WEEKS

Well-child / well-adolescent preventive visit • Infancy • Visit 5 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ length
- ☐ head circumference
- ☐ plotted

PHYSICAL EXAMINATION

- ☐ Surveillance exam
- ☐ muscle tone

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ DTWP-2
- ☐ IPV-2
- ☐ Hib-2
- ☐ PCV-2
- ☐ Rotavirus-2

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ Eyes tracked
- ☐ sound response

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Vocalises
- ☐ head control improving
- ☐ Maternal EPDS if indicated

ANTICIPATORY GUIDANCE GIVEN

- ☐ Feeding
- ☐ tummy time
- ☐ car/2-wheeler safety
- ☐ smoke-free home

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

14 WEEKS

Well-child / well-adolescent preventive visit • Infancy • Visit 6 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ length
- ☐ head circumference
- ☐ plotted

PHYSICAL EXAMINATION

- ☐ Surveillance exam
- ☐ hips recheck

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ DTWP-3
- ☐ IPV-3
- ☐ Hib-3
- ☐ PCV-3
- ☐ Rotavirus-3 (RV-3 not needed for monovalent RV1)

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ Eyes tracked
- ☐ localises sound

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Reaches & grasps
- ☐ laughs
- ☐ Behaviour surveillance

ANTICIPATORY GUIDANCE GIVEN

- ☐ Weaning readiness (start 6 mo)
- ☐ safe play
- ☐ heat/burn safety

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

6 MONTHS

Well-child / well-adolescent preventive visit • Infancy • Visit 7 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ length
- ☐ head circumference
- ☐ plotted
- ☐ note growth velocity

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ teeth eruption
- ☐ sits with support

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Hep B-3 (if not in combo)
- ☐ Influenza dose 1 (2 doses 4 wk apart first season)
- ☐ TCV may begin

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab/screen due routine (haemoglobin due at 12 mo)

VISION / HEARING / DENTAL

- ☐ Fix/follow both eyes
- ☐ turns to sound
- ☐ First teeth
- ☐ wipe gums
- ☐ plan dental visit by 12 mo

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ MEB screen
- ☐ sits
- ☐ transfers objects
- ☐ stranger awareness
- ☐ Child MEB screen begins (per AAP 2025)

ANTICIPATORY GUIDANCE GIVEN

- ☐ Start complementary feeding + continue breastfeeding
- ☐ iron-rich foods
- ☐ no screen time under 2 y
- ☐ drowning/bucket safety

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

9 MONTHS

Well-child / well-adolescent preventive visit • Infancy • Visit 8 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ length
- ☐ head circumference
- ☐ plotted

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ pincer grasp
- ☐ pulls to stand

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ MMR-1
- ☐ Influenza dose 2 (if first season)
- ☐ OPV (optional)

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ Cover test
- ☐ responds to name/sound
- ☐ Gums/first teeth
- ☐ fluoride per local water

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ DEVELOPMENTAL SCREEN (ASQ / TDSC / DDST)
- ☐ crawls
- ☐ babbles
- ☐ waves
- ☐ Developmental screen
- ☐ parent-child interaction observed

ANTICIPATORY GUIDANCE GIVEN

- ☐ Object-permanence play
- ☐ home-proofing
- ☐ choking hazards
- ☐ responsive feeding

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

12 MONTHS

Well-child / well-adolescent preventive visit • Infancy • Visit 9 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ length
- ☐ head circumference
- ☐ plotted

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ stands
- ☐ first steps
- ☐ anterior fontanelle

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Hep A-1
- ☐ annual Influenza

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Haemoglobin (anaemia screen) - high Indian prevalence
- ☐ TSH/others only if risk

VISION / HEARING / DENTAL

- ☐ Vision & hearing surveillance
- ☐ First dental visit due
- ☐ brush with fluoride toothpaste (rice-grain smear)

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ MEB surveillance
- ☐ jargon
- ☐ 1-2 words
- ☐ points
- ☐ MEB screen (12-month)

ANTICIPATORY GUIDANCE GIVEN

- ☐ Cup weaning
- ☐ family foods
- ☐ walking safety
- ☐ iron & vitamin D from diet/sun
- ☐ screen-time limits

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

15 MONTHS

Well-child / well-adolescent preventive visit • Early childhood • Visit 10 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ length
- ☐ head circumference
- ☐ plotted

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ gait
- ☐ play

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ MMR-2
- ☐ Varicella-1
- ☐ PCV booster

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ Vision/hearing tracked
- ☐ Brushing twice daily (supervised)

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Surveillance
- ☐ 3-5 words
- ☐ follows simple command
- ☐ Behaviour surveillance

ANTICIPATORY GUIDANCE GIVEN

- ☐ Language stimulation
- ☐ tantrum guidance
- ☐ medicine/poison lock-away
- ☐ helmet on 2-wheelers

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

18 MONTHS

Well-child / well-adolescent preventive visit • Early childhood • Visit 11 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ length
- ☐ head circumference
- ☐ plotted

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ runs
- ☐ stacks blocks

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ DTwP B1 (or DTaP), IPV B1, Hib B1
- ☐ Hep A-2 (killed, 6 mo after dose 1)

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ Vision/hearing surveillance
- ☐ Dental review

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ DEVELOPMENTAL SCREEN + AUTISM SCREEN (M-CHAT-R/F)
- ☐ M-CHAT-R/F (autism)
- ☐ MEB

ANTICIPATORY GUIDANCE GIVEN

- ☐ Toilet-training readiness
- ☐ screen time
- ☐ reading aloud
- ☐ discipline without harm

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

24 MONTHS (2 YEARS)

Well-child / well-adolescent preventive visit • Early childhood • Visit 12 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ standing height
- ☐ head circumference
- ☐ BMI (from 2 y)
- ☐ plotted

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ consider starting BP

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Annual Influenza
- ☐ catch-up as needed

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Repeat haemoglobin if prior anaemia or ongoing risk

VISION / HEARING / DENTAL

- ☐ Vision (instrument if available)
- ☐ hearing
- ☐ 6-monthly dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ AUTISM SCREEN (M-CHAT-R/F)
- ☐ 2-word phrases
- ☐ 50+ words
- ☐ MEB (24-month) + autism screen

ANTICIPATORY GUIDANCE GIVEN

- ☐ Balanced diet
- ☐ active play
- ☐ limit screens
- ☐ dental hygiene
- ☐ pedestrian road safety

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

30 MONTHS (2.5 YEARS)

Well-child / well-adolescent preventive visit • Early childhood • Visit 13 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI
- ☐ plotted

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Catch-up as needed

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ Vision/hearing surveillance
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ DEVELOPMENTAL SCREEN
- ☐ sentences
- ☐ understands prepositions
- ☐ Developmental screen (30-month)

ANTICIPATORY GUIDANCE GIVEN

- ☐ Preschool readiness
- ☐ social play
- ☐ sleep routine
- ☐ screen-time quality

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

3 YEARS

Well-child / well-adolescent preventive visit • Early childhood • Visit 14 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP (begins at 3 y)

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ BP measurement begins

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Catch-up as needed

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due (BP is a vital, not a lab)

VISION / HEARING / DENTAL

- ☐ VISION screening (chart) + HEARING screening
- ☐ 6-monthly dental
- ☐ fluoride

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Behaviour surveillance
- ☐ speech intelligibility
- ☐ SDQ if behaviour concern

ANTICIPATORY GUIDANCE GIVEN

- ☐ Preschool
- ☐ nutrition & obesity prevention
- ☐ outdoor play
- ☐ injury prevention

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

4 YEARS

Well-child / well-adolescent preventive visit • Early childhood • Visit 15 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ DTwP B2 (or DTaP), IPV B2, MMR-3, Varicella-2
- ☐ annual Influenza

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ VISION + HEARING screening
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Behaviour surveillance
- ☐ school readiness
- ☐ SDQ / behaviour screen if indicated

ANTICIPATORY GUIDANCE GIVEN

- ☐ School readiness
- ☐ healthy weight
- ☐ screen limits
- ☐ helmet & road safety
- ☐ good-touch/bad-touch

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

5 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 16 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ switch to IAP 5-18 charts

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Catch-up
- ☐ Typhoid booster if due

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ VISION + HEARING screening
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ School-performance surveillance
- ☐ Behaviour/attention screen if concern

ANTICIPATORY GUIDANCE GIVEN

- ☐ School adjustment
- ☐ nutrition
- ☐ physical activity 60 min/day
- ☐ sleep hygiene

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

6 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 17 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ posture/scoliosis awareness

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Catch-up as needed

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ VISION + HEARING screening
- ☐ 6-monthly dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Attention & learning surveillance
- ☐ SDQ / Vanderbilt (ADHD) if concern

ANTICIPATORY GUIDANCE GIVEN

- ☐ Reading
- ☐ screen time
- ☐ bullying
- ☐ road & water safety
- ☐ balanced diet

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

7 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 18 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Catch-up as needed

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ Vision/hearing surveillance
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Learning surveillance
- ☐ Behaviour screen if concern

ANTICIPATORY GUIDANCE GIVEN

- ☐ Physical activity
- ☐ limit sugary drinks
- ☐ sleep
- ☐ peer relationships

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

8 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 19 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ puberty-staging awareness

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Catch-up as needed

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm no routine lab due

VISION / HEARING / DENTAL

- ☐ VISION + HEARING screening
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Learning surveillance
- ☐ SDQ if concern

ANTICIPATORY GUIDANCE GIVEN

- ☐ Screen & sleep
- ☐ media safety
- ☐ nutrition
- ☐ injury prevention

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

9 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 20 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ Tanner staging

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ early-puberty check

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Consider HPV planning (9-14 y window)

LABORATORY & SCREENING (only if listed / indicated)

- ☐ LIPID / cholesterol screen (universal, once at 9-11 y)

VISION / HEARING / DENTAL

- ☐ Vision/hearing surveillance
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Learning & mood surveillance
- ☐ Mood surveillance
- ☐ begin adolescent rapport

ANTICIPATORY GUIDANCE GIVEN

- ☐ Puberty education
- ☐ nutrition
- ☐ activity
- ☐ screen & sleep
- ☐ emotional literacy

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

10 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 21 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ Tanner

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ scoliosis
- ☐ puberty

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Catch-up
- ☐ HPV may start (2-dose 9-14 y)

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Complete lipid screen if not done at 9 y

VISION / HEARING / DENTAL

- ☐ VISION + HEARING screening
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Mood & learning surveillance
- ☐ Mood surveillance
- ☐ psychosocial rapport

ANTICIPATORY GUIDANCE GIVEN

- ☐ Puberty
- ☐ peer pressure
- ☐ body image
- ☐ physical activity
- ☐ screen limits

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

11 YEARS

Well-child / well-adolescent preventive visit • Adolescence • Visit 22 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ Tanner

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ puberty

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Tdap
- ☐ HPV (2-dose if 9-14 y)
- ☐ annual Influenza

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Consider haemoglobin in menstruating/at-risk girls

VISION / HEARING / DENTAL

- ☐ Vision/hearing surveillance
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ HEEADSSS begins
- ☐ academic & social
- ☐ DEPRESSION screen (PHQ-2/PHQ-9A) begins
- ☐ SUBSTANCE-USE screen (CRAFFT/S2BI) begins

ANTICIPATORY GUIDANCE GIVEN

- ☐ Confidentiality framing
- ☐ puberty
- ☐ substance & tobacco
- ☐ mood
- ☐ sleep
- ☐ online safety

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

12 YEARS

Well-child / well-adolescent preventive visit • Adolescence • Visit 23 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ Tanner

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ puberty
- ☐ scoliosis

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Complete HPV/Tdap
- ☐ annual Influenza

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Haemoglobin if menstruating/at-risk

VISION / HEARING / DENTAL

- ☐ Vision/hearing surveillance
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ HEEADSSS
- ☐ identity & autonomy
- ☐ **DEPRESSION (annual) + SUICIDE-RISK screen (ASQ) + ANXIETY (SCARED) begins**
- ☐ substance use (CRAFFT)

ANTICIPATORY GUIDANCE GIVEN

- ☐ Mental-health rapport
- ☐ relationships
- ☐ consent
- ☐ body image
- ☐ social media
- ☐ RTA/helmet

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

13 YEARS

Well-child / well-adolescent preventive visit • Early childhood • Visit 24 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ Tanner

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Catch-up (HPV/Tdap/MMR/varicella as needed)

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Haemoglobin if at risk

VISION / HEARING / DENTAL

- ☐ Vision/hearing surveillance
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ HEEADSSS
- ☐ **Depression + suicide-risk + anxiety + substance use (annual)**

ANTICIPATORY GUIDANCE GIVEN

- ☐ Identity
- ☐ peer & risk behaviour
- ☐ nutrition
- ☐ screen & sleep
- ☐ safety

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

14 YEARS

Well-child / well-adolescent preventive visit • Early childhood • Visit 25 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ Tanner

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Govt UIP HPV for 14-y girls (2026)
- ☐ catch-up

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Haemoglobin if at risk

VISION / HEARING / DENTAL

- ☐ Vision/hearing surveillance
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ HEEADSSS
- ☐ academic stress
- ☐ **Depression + suicide-risk + anxiety + substance use (annual)**

ANTICIPATORY GUIDANCE GIVEN

- ☐ Exam stress & mental health
- ☐ substance
- ☐ sexuality & consent
- ☐ RTA
- ☐ digital wellbeing

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

15 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 26 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ Tanner

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ HPV single-dose acceptable 9-15 y (immunocompetent)
- ☐ catch-up

LABORATORY & SCREENING (only if listed / indicated)

- ☐ HIV screen once (15-21 y)
- ☐ STI if sexually active
- ☐ haemoglobin if at risk

VISION / HEARING / DENTAL

- ☐ HEARING (once 15-17 y)
- ☐ vision as needed
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ HEEADSSS
- ☐ **Depression + suicide-risk + anxiety + substance use (annual)**

ANTICIPATORY GUIDANCE GIVEN

- ☐ Autonomy
- ☐ risk reduction
- ☐ mental health
- ☐ contraception counselling as appropriate

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

16 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 27 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ Tanner

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Td (16-18 y)
- ☐ catch-up

LABORATORY & SCREENING (only if listed / indicated)

- ☐ STI if sexually active
- ☐ haemoglobin if at risk

VISION / HEARING / DENTAL

- ☐ Vision/hearing as needed
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ HEEADSSS
- ☐ future planning
- ☐ **Depression + suicide-risk + anxiety + substance use (annual)**

ANTICIPATORY GUIDANCE GIVEN

- ☐ Career/exam stress
- ☐ driving safety
- ☐ substance
- ☐ relationships
- ☐ sleep

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

17 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 28 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI percentile
- ☐ BP
- ☐ Tanner

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Catch-up as needed

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Repeat lipid (once 17-21 y)
- ☐ STI/HIV as indicated

VISION / HEARING / DENTAL

- ☐ HEARING (once 15-17 y)
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ HEEADSSS
- ☐ **Depression + suicide-risk + anxiety + substance use (annual)**

ANTICIPATORY GUIDANCE GIVEN

- ☐ Transition planning
- ☐ independence
- ☐ mental health
- ☐ substance
- ☐ safe driving

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

18 YEARS

Well-child / well-adolescent preventive visit • Middle childhood • Visit 29 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI
- ☐ BP

PHYSICAL EXAMINATION

- ☐ Exam
- ☐ adult transition

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Td if not given
- ☐ adult catch-up (HPV up to 26 y)

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Lipid if not done
- ☐ STI/HIV as indicated

VISION / HEARING / DENTAL

- ☐ Vision/hearing as needed
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ HEEADSSS
- ☐ transition to adult care
- ☐ **Depression + suicide-risk + anxiety + substance use (annual)**

ANTICIPATORY GUIDANCE GIVEN

- ☐ Transition-to-adult-care plan
- ☐ self-management
- ☐ mental health
- ☐ lifestyle

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

19-20 YEARS

Well-child / well-adolescent preventive visit • Adolescence • Visit 30 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI
- ☐ BP

PHYSICAL EXAMINATION

- ☐ Exam

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Adult catch-up as needed

LABORATORY & SCREENING (only if listed / indicated)

- ☐ STI/HIV as indicated
- ☐ lipid if not done 17-21 y

VISION / HEARING / DENTAL

- ☐ As needed
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Mental-health & psychosocial review
- ☐ Depression + anxiety + substance use (annual)

ANTICIPATORY GUIDANCE GIVEN

- ☐ Independence
- ☐ occupational health
- ☐ mental health
- ☐ substance
- ☐ reproductive health

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
- ☐ ENT / Audiology - if failed hearing screen, speech delay, recurrent otitis, or OSA symptoms
- ☐ Dental - if caries, delayed eruption, trauma, or no visit by 12 months
- ☐ **Child mental health - if any positive screen; SUICIDE-RISK positive = urgent, do not leave alone**
- ☐ Development / early intervention - if screen-positive delay, regression, or M-CHAT positive
- ☐ No referral required at this visit

SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____

21 YEARS

Well-child / well-adolescent preventive visit • Adolescence • Visit 31 of 31

Child's name: _____ DOB: _____

Date of visit: _____ Age today: _____ MRN / UHID: _____

MEASUREMENTS & GROWTH (record value, then tick)

- ☐ Weight
- ☐ height
- ☐ BMI
- ☐ BP

PHYSICAL EXAMINATION

- ☐ Final paediatric exam
- ☐ handover

IMMUNISATIONS DUE (IAP-ACVIP 2025 - verify against current chart)

- ☐ Confirm adult schedule complete

LABORATORY & SCREENING (only if listed / indicated)

- ☐ Confirm lipid & HIV done once in 18-21 y window

VISION / HEARING / DENTAL

- ☐ As needed
- ☐ Dental

DEVELOPMENT & MENTAL-BEHAVIOURAL HEALTH

- ☐ Confirm transition to adult provider
- ☐ Depression + anxiety + substance use
- ☐ transition summary

ANTICIPATORY GUIDANCE GIVEN

- ☐ Complete transfer to adult care with summary
- ☐ self-advocacy
- ☐ preventive-care literacy

REFERRALS CONSIDERED

- ☐ Ophthalmology - if abnormal red reflex, squint, leukocoria, or failed vision screen
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SIGN-OFF

Clinician name & signature: _____ Date: _____

Notes / actions: _____